

April 2024	Added clarifying language to criteria: If a member meets appropriate clinical scenarios with supporting provider documentation, approval may be granted for IUI treatments with oral medications and triggers.
December 2024	Added coverage criteria for ART and IVF (implementation 1/1/25). Added background information. Added and updated references.
February 2025	Updates to policy: removal of criteria relating to multiple blastocyte transfer update to criteria for non-covered indications in Assisted Reproductive Technologies
May 2025	Added CPT 55899 Unlisted Procedure, Male Genital System
January 1, 2026	Removal of gender affirmation as a qualifying criterion for infertility services. Moved general indications for infertility treatment and natural cycle indications from criteria section to Background section. Removed statement, "Cumulus cell removal is part of ICSI process". Clarification of Iatrogenic Infertility coverage for the High Plan. Added links relating to preimplantation genetic testing medical review and pre-authorization. Clarified semen analysis collection timeframe to 4 weeks apart. Clarified criteria language relating to recurrent pregnancy loss and aneuploidy. Removed limiting criteria relating to TESE for non-obstructive azoospermia. Clarification of limiting criteria for embryo and oocyte cryopreservation. Clarification of recommendations for a favorable eSET prognosis. Benefit language clarification for Iatrogenic Infertility coverage: Benefits are limited to one successful cycle of fertility preservation for Iatrogenic Infertility per covered person during the entire period of time enrolled for coverage under the healthcare plan. Successful fertility preservation cycle includes: 1. 8-10 mature oocytes obtained within no more than 3 attempts of retrieval or 2. 3-4 vials of sperm obtained within no more than 3 attempts of separate sperm extraction sessions. (ASRM, 2019)
June 2026	Corrected grammatical error with no impact on criteria coverage. Updated formatting for ease of use.